

Pembrolizumab + carboplatin/paclitaxel → AC

Neoadjuvant and adjuvant treatment for high-risk early triple-negative breast cancer

WHAT THIS REGIMEN IS FOR

For adults with locally advanced or early-stage triple-negative breast cancer at high risk of recurrence when this NCCP regimen is appropriate.

MEDICINES

Pembrolizumab

Carboplatin

Paclitaxel

Doxorubicin

Cyclophosphamide

TREATMENT JOURNEY

Cycles 1-4

Every 21 days:
pembrolizumab +
carboplatin on day 1;
paclitaxel on days 1, 8
and 15.

Cycles 5-8

Every 21 days:
pembrolizumab +
doxorubicin +
cyclophosphamide on
day 1.

Surgery

Definitive breast surgery
follows the neoadjuvant
phase when clinically
appropriate.

Adjuvant phase

Pembrolizumab
continues after surgery:
200 mg every 21 days
for 9 cycles or 400 mg
every 42 days for 5
cycles, according to the
treatment plan.

Throughout

Bloods and toxicity
review guide whether
treatment is given,
delayed or modified.

REGIMEN-SPECIFIC POINT

This is a multi-phase regimen. The medicines and side-effect emphasis change after cycle 4 and again after surgery.

MONITORING / SUPPORT

- Blood count, kidney/liver profile and glucose are checked regularly; thyroid testing is repeated during pembrolizumab treatment.
- Paclitaxel requires premedication according to the regimen/local protocol.
- Cardiac assessment may be required before the doxorubicin phase.

Where changes may show up

Patient-first anatomical guide · report the symptom rather than trying to diagnose the cause

Immune organs

Pembrolizumab can inflame lungs, bowel, liver, kidneys, skin, nerves and hormone glands. New symptoms matter even between cycles.

Blood + infection

Chemotherapy can lower white cells, haemoglobin and platelets. Fever or sudden illness needs immediate contact.

Nerves

Paclitaxel can cause tingling or numbness. Report symptoms before they become persistent or interfere with function.

Heart

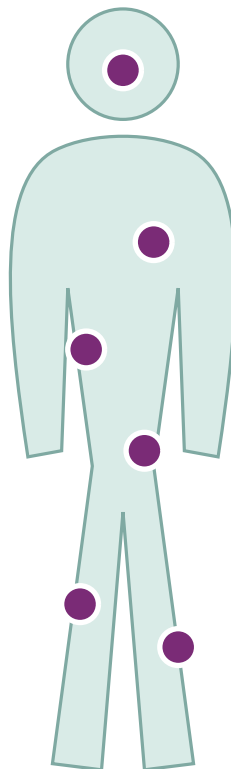
Doxorubicin can affect heart function. New chest symptoms or breathlessness need assessment.

Mouth + gut

Nausea, mouth soreness and bowel changes can occur. Follow supportive medicines and report persistent symptoms.

Skin + nails

Rash, nail changes and infusion reactions can occur; severe or blistering rash needs urgent review.



WHEN TO CALL

Call oncology urgently for fever, new breathlessness, persistent diarrhoea, jaundice, severe rash, significant bleeding, severe weakness or a new neurological symptom. For severe breathing difficulty, chest pain, collapse or seizure call 112/999.



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